

Dissociative Disorders

The essential feature is a sudden, temporary alteration in the normally integrative functions of consciousness, identity, or motor behavior. If the alteration occurs in consciousness, important personal events cannot be recalled. If it occurs in identity, either the individual's customary identity is temporarily forgotten and a new identity is assumed, or the customary feeling of one's own reality is lost and replaced by a feeling of unreality. If the alteration occurs in motor behavior, there is also a concurrent disturbance in consciousness or identity, as in the wandering that occurs during a Psychogenic Fugue.

Depersonalization Disorder has been included in the Dissociative Disorders because the feeling of one's own reality, an important component of identity, is lost. Some, however, question this inclusion because disturbance in memory is absent.

Although Sleepwalking Disorder has the essential feature of a Dissociative Disorder, it is classified among the Disorders Usually First Evident in Infancy, Childhood, or Adolescence.

300.12 Psychogenic Amnesia

The essential feature is a sudden inability to recall important personal information, an inability not due to an Organic Mental Disorder. The extent of the disturbance is too great to be explained by ordinary forgetfulness. The diagnosis is not made if the person travels to another locale and assumes a new identity, in which case the diagnosis is Psychogenic Fugue.

There are four types of disturbance in recall. In *localized* (or circumscribed) amnesia, the most common type, there is failure to recall all events occurring during a circumscribed period of time, usually the first few hours following a profoundly disturbing event. For example, the uninjured survivor of a car accident which killed his immediate family cannot recall anything that happened from the time of the accident until two days later. Somewhat less common is *selective* amnesia, a failure to recall some, but not all, of the events occurring during a circumscribed period of time. In the illustration above, the uninjured survivor might recall making the funeral arrangements, but not recall extensive simultaneous discussions with family members. The least common types of disturbance in recall are *generalized* amnesia, in which failure of recall encompasses the individual's entire life, and *continuous* amnesia, in which the individual cannot recall events subsequent to a specific time up to and including the present.

During an ongoing amnesic episode, perplexity, disorientation, and purposeless wandering may occur. When the period of time for which there is amnesia is in the past, the person is usually aware of the disturbance in recall.

Associated features. During the amnesic period there may be indifference toward the memory disturbance. Post-traumatic Stress Disorder may also be present.

Age at onset and sex ratio. Military sources provide many clinical reports describing the disorder in young males during war. The disorder is most often observed in adolescent and young adult females, but rarely in the elderly.

Course and predisposing factors. Amnesia begins suddenly, usually following severe psychosocial stress. The stress often involves a threat of physical injury or death. In other instances the stress is due to the unacceptability of certain impulses or acts, such as an extramarital affair. In still other instances the individual may be in a subjectively intolerable life situation, such as abandonment by a spouse.

Termination of the amnesia is typically abrupt. Recovery is complete and recurrences are rare.

Impairment. The degree of impairment varies from mild to severe in proportion to the duration of the amnesic episode and the importance of forgotten events to the individual's social functioning. The impairment is usually minimal and temporary, since rapid recovery is the rule.

Complications. None.

Prevalence. The condition is apparently rare under normal circumstances; it is more common in wartime or during natural disasters.

Familial pattern. No information.

Differential diagnosis. In **Organic Mental Disorders** there is usually a memory disturbance whose onset has no relationship to stress and that is more marked for recent than for remote events. Memory impairment caused by organic factors usually disappears very slowly, if at all; full return of memory is rare. Furthermore, attention deficits, a clouding of consciousness, and disturbances of affect are frequently present.

In **Substance-induced Intoxication** there can be "blackouts" with failure to recall events that occurred during the intoxication. The organic factor (the substance taken) and the failure to achieve full return of memory clearly distinguish it from Psychogenic Amnesia.

In **Alcohol Amnesic Disorder**, short-term (not immediate) memory is impaired, i.e., events can be recalled immediately after they occur, but not after the passage of a few minutes. This type of memory disturbance is not seen in Psychogenic Amnesia. In addition, blunted affect, confabulation, and lack of awareness of the memory impairment are common in Alcohol Amnesic Disorder.

In **postconcussion amnesia**, the disturbance of recall, though circumscribed,

is often retrograde, encompassing a period of time before the head trauma, whereas in Psychogenic Amnesia the disturbance of recall is almost always anterograde. Retrograde amnesia following head trauma can usually be distinguished from Psychogenic Amnesia by diagnostic use of hypnosis or an amytal interview; prompt recovery of the lost memories suggests a psychogenic basis for the disturbance.

In **epilepsy**, the memory impairment is sudden in onset, motor abnormalities are usually present during the episode, and repeated EEGs typically reveal anomalies.

In **catatonic stupor**, mutism may suggest Psychogenic Amnesia, but failure of recall is nearly always absent, and there usually are other characteristic catatonic symptoms, such as rigidity, posturing, and negativism.

Malingering involving simulated amnesia presents a particularly difficult diagnostic dilemma. Attention to the possibility that the amnesia is feigned plus careful questioning under hypnosis or during an amytal interview should help to resolve the dilemma.

Diagnostic criteria for Psychogenic Amnesia

A. Sudden inability to recall important personal information that is too extensive to be explained by ordinary forgetfulness.

B. The disturbance is not due to an Organic Mental Disorder (e.g., blackouts during Alcohol Intoxication).

300.13 Psychogenic Fugue

The essential feature is sudden, unexpected travel away from home or customary work locale with assumption of a new identity and an inability to recall one's previous identity. Perplexity and disorientation may occur. Following recovery there is no recollection of events that took place during the fugue. The diagnosis is not made in the presence of an Organic Mental Disorder.

In some cases the disorder may be manifested by the assumption of a completely new identity during the fugue, usually marked by more gregarious and uninhibited traits than characterized the former personality, which typically is quiet and altogether ordinary. In such instances the individual may give himself or herself a new name, take up a new residence, and engage in complex social activities that are well-integrated and do not suggest the presence of a mental disorder. In most cases, however, the fugue is less elaborate, and consists of little more than brief, apparently purposeful travel. Social contacts in these cases are minimal or even avoided; the new identity, while present, is incomplete. Occasionally there are outbursts of violence against another person or property. In all cases of fugue, however, the individual's travel and behavior must appear more purposeful than the confused wandering that may be seen in Psychogenic Amnesia.

Associated features. No information.

Age at onset. Variable.

Predisposing factors and course. Heavy alcohol use may predispose to the development of the disorder. Psychogenic Fugue typically follows severe psychosocial stress, such as marital quarrels, personal rejections, military conflict, or natural disaster. Usually the fugue is of brief duration—hours to days—and involves a limited amount of travel; more rarely, it continues for many months and involves complex but unobtrusive travel over thousands of miles and across numerous national borders. Ultimately, rapid recovery occurs; recurrences are rare.

Impairment. The degree of impairment varies with the duration of the fugue and the extent to which it causes subsequent social distress to the individual and his or her associates. When violent behavior has occurred, the social, legal, and other personal sequelae depend on the nature of the violent act. In most cases impairment is minimal and transient.

Complications. None.

Prevalence. Although apparently rare, the disorder is most common in wartime, or in the wake of a natural disaster.

Sex ratio and familial pattern. No information.

Differential diagnosis. **Organic Mental Disorders** usually involve a disturbance of memory more marked for recent than for remote events; the memory disturbance is not isolated and disappears slowly, if at all; memory rarely is fully restored. Disturbances of attention, clouding of consciousness, and affective disturbances are usually present but unexpected travel is unusual. If travel occurs as part of an Organic Mental Disorder, it is usually not of a complex, purposive, or socially adaptive nature and appears to be mere wandering.

When **temporal lobe epilepsy** involves travel, motoric activity is usually simple rather than complex and there is no assumption of a new identity. Affect is dysphoric. Typically, temporal lobe epilepsy is not precipitated by psychosocial stress.

In **Psychogenic Amnesia**, sudden failure to recall important personal events, including one's personal identity, occurs; but purposeful travel and the assumption of a new identity, partial or complete, are not present.

Malingering, in which there is feigned inability to recall one's previous activity and identity, is exceedingly difficult to distinguish from Psychogenic Fugue. Careful questioning under hypnosis or during an amytal interview can be useful.

Diagnostic criteria for Psychogenic Fugue

- A. Sudden unexpected travel away from one's home or customary place of work, with inability to recall one's past.
- B. Assumption of a new identity (partial or complete).
- C. The disturbance is not due to an Organic Mental Disorder.

300.14 Multiple Personality

The essential feature is the existence within the individual of two or more distinct personalities, each of which is dominant at a particular time. Each personality is a fully integrated and complex unit with unique memories, behavior patterns, and social relationships that determine the nature of the individual's acts when that personality is dominant. Transition from one personality to another is sudden and often associated with psychosocial stress.

Usually the original personality has no knowledge or awareness of the existence of any of the other personalities (subpersonalities). When there are more than two subpersonalities in one individual, each is aware of the others to varying degrees. The subpersonalities may not know each other or be constant companions. At any given moment one personality will interact verbally with the external environment, but none or any number of the other personalities may actively perceive (i.e., "listen in on") all that is going on.

The original personality and all of the subpersonalities are aware of lost periods of time. "They" will usually admit to this if asked, but will seldom volunteer this information.

The individual personalities are nearly always quite discrepant and frequently seem to be opposites. For example, a quiet, retiring spinster may alternate with a flamboyant, promiscuous bar habitué on certain nights. Usually one of the personalities over the course of the disorder is dominant.

Associated features. One or more of the personalities may function with a reasonable degree of adaptation (e.g., be gainfully employed) while alternating with another personality that is clearly maladapted or has a specific, separate, mental disorder. Studies have demonstrated that various subpersonalities in the same individual may have different responses to physiological and psychological measurements.

One or more of the subpersonalities may report being of the opposite sex, of a different race or age, or from a different family than the original personality. Each subpersonality, however, displays behaviors characteristic of its stated age, which is usually younger than the actual age.

One or more of the personalities may be aware of hearing or having heard the voice(s) of one or more of the other personalities or may report having talked with or engaged in activities with one or more of the other personalities. These internal conversations and the belief that one has engaged in activities with another personality when the latter is actually a dissociated aspect of the

original personality must be differentiated from other forms of hallucinatory and delusional experiences.

The subpersonalities often exist in groups of two or three, all of whom represent the same period of life (e.g., adolescence). When this occurs, one or more of these subpersonalities tends to have the role of protector of another member(s) of the group.

Psychosocial stress most often precipitates the transition from one personality to another; hypnosis may also effect this change. Usually transitions occur in a dramatic manner.

Most often, the subpersonalities have proper names, usually different from the first name, and sometimes from both the first and last names, of the original personality. Occasionally one or more subpersonalities are unnamed.

Somatoform Disorders and Psychological Factors Affecting Physical Condition apparently are common in individuals with Multiple Personality.

Age at onset. Onset of Multiple Personality may be in early childhood or later. The disorder is rarely diagnosed until adolescence.

Course. The course tends to be more chronic than in the other Dissociative Disorders.

Impairment. The degree of impairment varies from moderate to severe, depending on the number, nature, and persistence of the various subpersonalities. Impairment is greater than in the other Dissociative Disorders and recovery typically is less complete.

Complications. Transient psychotic episodes, Psychosexual Disorders and Disorders of Impulse Control Not Elsewhere Classified, may be complications.

Predisposing factors. Child abuse and other forms of severe emotional trauma in childhood may be predisposing factors.

Prevalence. The disorder is apparently extremely rare.

Sex ratio. Multiple Personality is most often diagnosed in late adolescent and young adult females.

Familial pattern. No information.

Differential diagnosis. Psychogenic Fugue and Psychogenic Amnesia may be confused with Multiple Personality, but do not present its characteristic repeated shifts of identity and usually are limited to a single, brief episode. Also, in both Psychogenic Amnesia and Psychogenic Fugue, awareness of the original personality is absent. Complex social activities, memories, behavior patterns, and friendships are not present in Psychogenic Amnesia and are uncommon in Psychogenic Fugue.

Psychotic disorders such as **Schizophrenic Disorders** may be confused with Multiple Personality because the individual reports hearing or talking with the voices of other personalities.

Malingering can present a difficult diagnostic dilemma. The presence of secondary gain suggests Malingering. Hypnosis or amytal interview may be of help in resolving especially difficult cases.

Diagnostic criteria for Multiple Personality

A. The existence within the individual of two or more distinct personalities, each of which is dominant at a particular time.

B. The personality that is dominant at any particular time determines the individual's behavior.

C. Each individual personality is complex and integrated with its own unique behavior patterns and social relationships.

300.60 Depersonalization Disorder

The essential feature is the occurrence of one or more episodes of depersonalization that cause social or occupational impairment. The diagnosis is not made when the symptom of depersonalization is secondary to any other disorder. (Mild depersonalization, without significant impairment, is estimated to occur at some time in 30%-70% of young adults.)

The symptom of depersonalization involves an alteration in the perception or experience of the self so that the usual sense of one's own reality is temporarily lost or changed. This is manifested by a sensation of self-estrangement or unreality, which may include the feeling that one's extremities have changed in size, or the experience of seeming to perceive oneself from a distance. In addition, the individual may feel "mechanical" or as though in a dream. Various types of sensory anesthetics and a feeling of not being in complete control of one's actions, including speech, are often present. All of these feelings are ego-dystonic, and the individual maintains grossly intact reality testing.

The onset of depersonalization is rapid; its disappearance is more gradual.

Associated features. Derealization is frequently present. This is manifested by a strange alteration in the perception of one's surroundings so that a sense of the reality of the external world is lost. A perceived change in the size or shape of objects in the external world is common. People may be perceived as dead or mechanical.

Other common associated features include dizziness, depression, obsessive ruminations, anxiety, fear of going insane, and a disturbance in the subjective sense of time. There is often the feeling that recall is difficult or slow.

Age at onset. The disorder usually begins in adolescence. Onset after the age of forty is extremely rare.

Course. The course is generally chronic and marked by remissions and exacerbations. Most often the exacerbations occur when there is mild anxiety or depression.

Impairment. The degree of impairment is usually minimal, but may be exacerbated by the presence of associated features such as anxiety or fear of insanity.

Complications. Hypochondriasis may be a complication of this disorder.

Predisposing factors. Fatigue, recovery from Substance Intoxication, hypnosis, meditation, physical pain, anxiety, depression, and severe stress, such as military combat or an auto accident, predispose to episodes of Depersonalization Disorder.

Prevalence, familial pattern, and sex ratio. No information.

Differential diagnosis. The symptom of depersonalization, even if recurrent, that does not cause any social or occupational impairment, must be distinguished from Depersonalization Disorder.

In **Schizophrenia, Affective Disorders, Organic Mental Disorders** (especially **Intoxication or Withdrawal**), **Anxiety Disorders, Personality Disorders**, and **epilepsy**, depersonalization may be a symptom. In such cases, the additional diagnosis of Depersonalization Disorder is not made.

Diagnostic criteria for Depersonalization Disorder

A. One or more episodes of depersonalization sufficient to produce significant impairment in social or occupational functioning.

B. The symptom is not due to any other disorder, such as Schizophrenia, Affective Disorder, Organic Mental Disorder, Anxiety Disorder, or epilepsy.

300.15 Atypical Dissociative Disorder

This is a residual category to be used for individuals who appear to have a Dissociative Disorder but do not satisfy the criteria for a specific Dissociative Disorder. Examples include trance-like states, derealization unaccompanied by depersonalization, and those more prolonged dissociated states that may occur in persons who have been subjected to periods of prolonged and intense coercive persuasion (brainwashing, thought reform, and indoctrination while the captive of terrorists or cultists).

Psychosexual Disorders

The name for this diagnostic class emphasizes that psychological factors are assumed to be of major etiological significance in the development of the disorders listed here. Disorders of sexual functioning that are caused exclusively by organic factors, even though they may have psychological consequences, are not listed in this classification. For example, impotence due to spinal-cord injury is coded on Axis III as a physical disorder, and the psychological reaction to that condition could be coded as an Adjustment Disorder, or some other suitable category, on Axis I.

The Psychosexual Disorders are divided into four groups. The Gender Identity Disorders are characterized by the individual's feelings of discomfort and inappropriateness about his or her anatomic sex and by persistent behaviors generally associated with the other sex. The Paraphilias are characterized by arousal in response to sexual objects or situations that are not part of normative arousal-activity patterns and that in varying degrees may interfere with the capacity for reciprocal affectionate sexual activity. The Psychosexual Dysfunctions are characterized by inhibitions in sexual desire or the psychophysiological changes that characterize the sexual response cycle. Finally, there is a residual class of Other Psychosexual Disorders that has two categories: Ego-dystonic Homosexuality and a final residual category, Psychosexual Disorders Not Elsewhere Classified.

GENDER IDENTITY DISORDERS

The essential feature of the disorders included in this subclass is an incongruence between anatomic sex and gender identity. Gender identity is the sense of knowing to which sex one belongs, that is, the awareness that "I am a male," or "I am a female." Gender identity is the private experience of gender role, and gender role is the public expression of gender identity. Gender role can be defined as everything that one says and does, including sexual arousal, to indicate to others or to the self the degree to which one is male or female.

Disturbance in gender identity is rare, and should not be confused with the far more common phenomena of feelings of inadequacy in fulfilling the expectations associated with one's gender role. An example would be an individual who perceives himself or herself as being sexually unattractive yet experiences himself or herself unambiguously as a man or woman in accordance with his or her anatomic sex.

302.5x Transsexualism

The essential features of this heterogeneous disorder are a persistent sense of discomfort and inappropriateness about one's anatomic sex and a persistent wish

to be rid of one's genitals and to live as a member of the other sex. The diagnosis is made only if the disturbance has been continuous (not limited to periods of stress) for at least two years, is not due to another mental disorder, such as Schizophrenia, and is not associated with physical intersex or genetic abnormality.

Individuals with this disorder usually complain that they are uncomfortable wearing the clothes of their own anatomic sex; frequently this discomfort leads to cross-dressing (dressing in clothes of the other sex). Often they choose to engage in activities that in our culture tend to be associated with the other sex. These individuals often find their genitals repugnant, which may lead to persistent requests for sex reassignment by surgical or hormonal means.

To varying degrees, the behavior, dress, and mannerisms are those of the other sex. With cross-dressing, hormonal treatment, and electrolysis, a few males with the disorder will appear relatively indistinguishable from members of the other sex. However, the anatomic sex of most males and females with the disorder is quite apparent to the alert observer.

Associated features. Generally there is moderate to severe coexisting personality disturbance. Frequently there is considerable anxiety and depression, which the individual may attribute to inability to live in the role of the desired sex.

Course and subtypes. The disorder is subdivided according to the predominant prior sexual history, which is coded in the fifth digit as 1 = asexual, 2 = homosexual (same anatomic sex), 3 = heterosexual (opposite anatomic sex), and 0 = unspecified. In the first, "asexual," the individual reports never having had strong sexual feelings. Often there is the additional history of little or no sexual activity or pleasure derived from the genitals. In the second group, "homosexual," a predominantly homosexual (object choice is same anatomic sex) arousal pattern preceding the onset of the Transsexualism is acknowledged, although often such individuals will deny that the behavior is homosexual because of their conviction that they are "really" of the other sex. In the third group, "heterosexual," the individual claims to have had an active heterosexual life.

Without treatment, the course of all three types is chronic and unremitting. Since surgical sex reassignment is a recent development, the long-term course of the disorder with this treatment is unknown.

Individuals who have female-to-male Transsexualism appear to represent a more homogeneous group than those who have male-to-female Transsexualism in that they are more likely to have a history of homosexuality and to have a more stable course, with or without treatment.

Age at onset. Individuals who develop Transsexualism often evidenced gender identity problems as children. However, some assert that although they were secretly aware of their gender problem, it was not evident to their family and friends. The age at which the full syndrome appears for those with the "asexual" or "homosexual" course is most often in late adolescence or early

adult life. In individuals with the "heterosexual" course, the disorder may have a later onset.

Impairment and complications. Frequently social and occupational functioning are markedly impaired, partly because of associated psychopathology and partly because of problems encountered in attempting to live in the desired gender role. Depression is common, and can lead to suicide attempts. In rare instances males may mutilate their genitals.

Predisposing factors. Extensive, pervasive, childhood femininity in a boy or childhood masculinity in a girl increases the likelihood of Transsexualism. Transsexualism seems always to develop in the context of a disturbed parent-child relationship. Some cases of Transvestism evolve into Transsexualism.

Prevalence. The disorder is apparently rare.

Sex ratio. Males are more common than females among people who seek help at clinics specializing in the treatment of this disorder. The ratio varies from as high as 8:1 to as low as 2:1.

Familial pattern. No information.

Differential diagnosis. In **effeminate homosexuality** the individual displays behaviors characteristic of the opposite sex. However, such individuals have no desire to be of the other anatomic sex. In **physical intersex** the individual may have a disturbance in gender identity. However, the presence of abnormal sexual structures rules out the diagnosis of Transsexualism.

Other individuals with a disturbed gender identity may, in isolated periods of stress, wish to belong to the other sex and to be rid of their own genitals. In such cases the diagnosis Atypical Gender Identity Disorder should be considered, since the diagnosis of Transsexualism is made only when the disturbance has been continuous for at least two years. In **Schizophrenia**, there may be delusions of belonging to the other sex, but this is rare. The insistence by an individual with Transsexualism that he or she is of the other sex is, strictly speaking, not a delusion since what is invariably meant is that the individual *feels like* a member of the other sex rather than a true belief that he or she *is* a member of the other sex.

In both **Transvestism** and **Transsexualism** there may be cross-dressing. However, in Transvestism that has not evolved into Transsexualism there is no wish to be rid of one's own genitals.

Diagnostic criteria for Transsexualism

- A. Sense of discomfort and inappropriateness about one's anatomic sex.
- B. Wish to be rid of one's own genitals and to live as a member of the other sex.
- C. The disturbance has been continuous (not limited to periods of stress) for at least two years.

D. Absence of physical intersex or genetic abnormality.

E. Not due to another mental disorder, such as Schizophrenia.

Fifth-digit code numbers and subclassification. The predominant prior sexual history is recorded in the fifth digit as:

1 = asexual

2 = homosexual (same anatomic sex)

3 = heterosexual (other anatomic sex)

0 = unspecified

302.60 Gender Identity Disorder of Childhood

The essential features are a persistent feeling of discomfort and inappropriateness in a child about his or her anatomic sex and the desire to be, or insistence that he or she is, of the other sex. In addition, there is a persistent repudiation of the individual's own anatomic attributes. This is not merely the rejection of stereotypical sex role behavior as, for example, in "tomboyishness" in girls or "sissyish" behavior in boys, but rather a profound disturbance of the normal sense of maleness or femaleness.

Girls with this disorder regularly have male peer groups, an avid interest in sports and rough-and-tumble play, and a lack of interest in playing with dolls or playing "house" (unless playing the father or another male role). More rarely, a girl with this disorder claims that she will grow up to become a man (not merely in role), that she is biologically unable to become pregnant, that she will not develop breasts, or that she has, or will grow, a penis.

Boys with this disorder invariably are preoccupied with female stereotypical activities. They may have a preference for dressing in girls' or women's clothes, or may improvise such items from available material when genuine articles are unavailable. (The cross-dressing never causes sexual excitement.) They often have a compelling desire to participate in the games and pastimes of girls. Dolls are often the favorite toy, and girls are regularly the preferred playmates. When playing "house," the role of a female is typically adopted. Rough-and-tumble play or sports are regularly avoided. Gestures and actions are often judged against a standard of cultural stereotype to be feminine, and the boy is invariably subjected to male peer group teasing and rejection, which rarely occurs among girls until adolescence. In rare cases a boy with this disorder claims that his penis or testes are disgusting or will disappear, or that it would be better not to have a penis or testes.

Some children refuse to attend school because of teasing or pressure to dress in attire stereotypical of their sex. Most children with this disorder deny being disturbed by it except as it brings them into conflict with the expectations of their family or peers.

Associated features. Some of these children, particularly girls, show no

other signs of psychopathology. Others may display serious signs of disturbance, such as phobias and persistent nightmares.

Age at onset and course. Three-fourths of the boys who cross-dress begin to do so before their fourth birthday; playing with dolls begins during the same period. Social ostracism increases during the early grades of school, and social conflict is significant at about age seven or eight. During the later grade-school years, grossly feminine behavior may lessen. As yet undetermined proportion of boys, perhaps one-third to one-half, become aware of a homosexual orientation during adolescence.

For females the age at onset is also early, but most begin to acquiesce to social pressure during late childhood or adolescence and give up an exaggerated insistence on male activities and attire. A minority retain a masculine identification and some of these develop a homosexual arousal pattern.

Complications. In a small number of cases, the disorder becomes continuous with Transsexualism.

Impairment. Peer relations with members of the same sex are absent or difficult to establish. The amount of impairment varies from none to extreme, and is related to the degree of underlying psychopathology and the reaction of peers and family to the individual's behavior.

Prevalence. The disorder is apparently rare.

Sex ratio and familial pattern. No information.

Predisposing factors. Extreme, excessive, and prolonged physical and emotional closeness between the infant and the mother and a relative absence of the father during the earliest years may contribute to the development of this disorder in the male. Females who later develop this disorder have mothers who were apparently unavailable to them at a very early age, either psychologically or physically, because of illness or abandonment; the girl seems to make a compensatory identification with the father, which leads to the adoption of a male gender identity.

Differential diagnosis. Children whose behavior merely does not fit the cultural stereotype of masculinity or femininity should not be given this diagnosis unless the full syndrome is present. **Physical abnormalities of the sex organs** are rarely associated with Gender Identity Disorder; when they are present, the physical disorder should be noted on Axis III.

Diagnostic criteria for Gender Identity Disorder of Childhood

For females:

- A. Strongly and persistently stated desire to be a boy, or insistence that she is a boy (not merely a desire for any perceived cultural advantages from being a boy).

B. Persistent repudiation of female anatomic structures, as manifested by at least one of the following repeated assertions:

- (1) that she will grow up to become a man (not merely in role)
- (2) that she is biologically unable to become pregnant
- (3) that she will not develop breasts
- (4) that she has no vagina
- (5) that she has, or will grow, a penis

C. Onset of the disturbance before puberty. (For adults and adolescents, see Atypical Gender Identity Disorder.)

For males:

A. Strongly and persistently stated desire to be a girl, or insistence that he is a girl.

B. Either (1) or (2):

(1) persistent repudiation of male anatomic structures, as manifested by at least one of the following repeated assertions:

- (a) that he will grow up to become a woman. (not merely in role)
- (b) that his penis or testes are disgusting or will disappear
- (c) that it would be better not to have a penis or testes

(2) preoccupation with female stereotypical activities as manifested by a preference for either cross-dressing or simulating female attire, or by a compelling desire to participate in the games and pastimes of girls

C. Onset of the disturbance before puberty. (For adults and adolescents, see Atypical Gender Identity Disorder.)

302.85 Atypical Gender Identity Disorder

This is a residual category for coding disorders in gender identity that are not classifiable as a specific Gender Identity Disorder.

PARAPHILIAS

The essential feature of disorders in this subclass is that unusual or bizarre imagery or acts are necessary for sexual excitement. Such imagery or acts tend to be insistently and involuntarily repetitive and generally involve either: (1) preference for use of a nonhuman object for sexual arousal, (2) repetitive sexual activity with humans involving real or simulated suffering or humiliation, or (3) repetitive sexual activity with nonconsenting partners. In other classifications these disorders are referred to as Sexual Deviations. The term Paraphilia is

preferable because it correctly emphasizes that the deviation (para) is in that to which the individual is attracted (philia).

The imagery in a Paraphilia, such as simulated bondage, may be playful and harmless and acted out with a mutually consenting partner. More likely it is not reciprocated by the partner, who consequently feels erotically excluded or superfluous to some degree. In more extreme form, paraphiliac imagery is acted out with a nonconsenting partner, and is noxious and injurious to the partner (as in severe Sexual Sadism) or to the self (as in Sexual Masochism).

Since paraphiliac imagery is necessary for erotic arousal, it must be included in masturbatory or coital fantasies, if not actually acted out alone or with a partner and supporting cast or paraphernalia. In the absence of paraphiliac imagery there is no relief from nonerotic tension, and sexual excitement or orgasm is not attained.

The imagery in a paraphiliac fantasy or the object of sexual excitement in a Paraphilia is frequently the stimulus for sexual excitement in individuals without a Psychosexual Disorder. For example, women's undergarments and imagery of sexual coercion are sexually exciting for many men; they are paraphiliac only when they become necessary for sexual excitement.

The Paraphilias included here are, by and large, conditions that traditionally have been specifically identified by previous classifications. Some of them are extremely rare; others are relatively common. Because some of these disorders are associated with nonconsenting partners, they are of legal and social significance. Individuals with these disorders tend not to regard themselves as ill, and usually come to the attention of mental health professionals only when their behavior has brought them into conflict with society.

The specific Paraphilias described here are: (1) Fetishism, (2) Transvestism, (3) Zoophilia, (4) Pedophilia, (5) Exhibitionism, (6) Voyeurism, (7) Sexual Masochism, and (8) Sexual Sadism. Finally, there is a residual category, Atypical Paraphilia, for noting the many other Paraphilias that exist but that have not been sufficiently described to date to warrant inclusion as specific categories.

Paraphilias may be multiple or may coexist with other mental disorders, such as Schizophrenia or various Personality Disorders. In such cases multiple diagnoses should be made.

Associated features. Frequently these individuals assert that the behavior causes them no distress and that their only problem is the reaction of others to their behavior. Others admit to guilt, shame, and depression at having to engage in an unusual sexual activity that is socially unacceptable. There is often impairment in the capacity for reciprocal affectionate sexual activity, and psychosexual dysfunctions are common. Personality disturbances, particularly emotional immaturity, are also frequent.

Impairment. Social and sexual relationships may suffer if others, such as a spouse (many of these individuals are married), become aware of the unusual sexual behavior. In addition, if the individual engages in sexual activity with a partner who refuses to cooperate in the unusual behavior, such as fetishistic or sadistic behavior, sexual excitement may be inhibited and the relationship may

suffer. In rare instances the unusual behavior may become the major activity in the individual's life, such as the collection of fetishes or voyeuristic acts.

Complications. In Zoophilia physical harm may result from sexual activity with animals. In Sexual Masochism, the individual may inflict serious physical damage on himself or herself. Paraphilias involving another person, particularly Voyeurism, Exhibitionism, and Pedophilia, often lead to arrest and incarceration. Sexual offenses against children constitute a significant proportion of all reported criminal sex acts. Individuals with Exhibitionism make up about one-third of all apprehended sex offenders.

Predisposing factors. With the exception of Transvestism (see p. 269), predisposing factors are unknown.

Prevalence. The disorders are apparently rare.

Sex ratio. Virtually all reported cases have been in males, with the exception of Sexual Sadism and Sexual Masochism, which, however, occur far more commonly in males. Although no cases of Voyeurism in women have been reported in the literature, some clinicians claim to know of such cases.

Familial pattern. No information.

302.81 **Fetishism**

The essential feature is the use of nonliving objects (fetishes) as a repeatedly preferred or exclusive method of achieving sexual excitement. The diagnosis is not made when the fetishes are limited to articles of female clothing used in cross-dressing, as in Transvestism, or when the object is sexually stimulating because it has been designed for that purpose, e.g., a vibrator.

Sexual activity may involve the fetish alone, such as masturbation into a shoe, or the fetish may be integrated into sexual activities with a human partner. In the latter situation the fetish is required or strongly preferred for sexual excitement, and in its absence there may be erectile failure in males.

Fetishes tend to be articles of clothing, such as female undergarments, shoes, and boots, or, more rarely, parts of the human body, such as hair or nails. The fetish is often associated with someone with whom the individual was intimately involved during childhood, most often a caretaker.

Age at onset. Usually the disorder begins by adolescence, although the fetish may have been endowed with special significance earlier, in childhood. Once established, the disorder tends to be chronic.

Differential diagnosis. Nonpathological sexual experimentation can involve sexual arousal by nonhuman objects, but this stimulus for sexual excitement is neither persistently preferred nor required.

In **Transvestism** the sexual arousal is limited to articles of female clothing used in cross-dressing. Although Transvestism could be considered fetishistic cross-dressing, the additional diagnosis of Fetishism should not be made.

Diagnostic criteria for Fetishism

A. The use of nonliving objects (fetishes) is a repeatedly preferred or exclusive method of achieving sexual excitement.

B. The fetishes are not limited to articles of female clothing used in cross-dressing (Transvestism) or to objects designed to be used for the purpose of sexual stimulation (e.g., vibrator).

302.30 Transvestism

The essential feature is recurrent and persistent cross-dressing by a heterosexual male that during at least the initial phase of the illness is for the purpose of sexual excitement. Interference with the cross-dressing results in intense frustration. This diagnosis is not made in those rare instances in which the disturbance has evolved into Transsexualism.

Transvestic phenomena range from occasional solitary wearing of female clothes to extensive involvement in a transvestic subculture. Usually more than one article of women's clothing is involved, and the man may dress entirely as a woman. The degree to which the cross-dressed individual appears as a woman varies, depending on mannerisms, body habitus, and cross-dressing skill. When not cross-dressed, he is usually unremarkably masculine. Although the basic preference is heterosexual, rarely has the individual had sexual experience with several women, and occasional homosexual acts may occur.

Age at onset and course. Cross-dressing typically begins in childhood or early adolescence. In some cases the cross-dressing is not done in public until adulthood. The initial experience may involve partial or total cross-dressing; when it is partial, it often progresses to total. A favored article of clothing may become erotic in itself and may habitually be used first in masturbation, and later in intercourse. In some individuals sexual arousal by the clothing tends to disappear, although the cross-dressing continues as an antidote to anxiety. Cross-dressing, although intermittent in the beginning, often becomes more frequent, and may become habitual. A small number of individuals with Transvestism, as the years pass, want to dress and live permanently as women, and the disorder may evolve into Transsexualism.

Predisposing factors. According to the folklore of individuals with this condition, a "petticoat punishment," the punishment of humiliating a boy by dressing him in the clothes of a girl, is common in the history of individuals who later develop this disorder.

Differential diagnosis. In **Transsexualism** there is a persistent wish to be rid of one's own genitals and to live as a member of the other sex, and there is never any sexual excitement with cross-dressing. The individual with Transvestism considers himself to be basically male, whereas the anatomically male Transsexual has a female sexual identity. In those rare instances when Transvestism evolves into Transsexualism, the diagnosis of Transvestism is changed to Transsexualism.

Cross-dressing for the relief of tension or gender discomfort may be done without directly causing sexual excitement. This should not be diagnosed as Transvestism; the diagnosis of Atypical Gender Identity Disorder should be considered. In **male homosexuality** there may be occasional cross-dressing to attract another male or to masquerade in theatrical fashion as a woman. However, the act of cross-dressing does not cause sexual arousal. In **female impersonators**, unless Transvestism is also present, the act of cross-dressing does not cause sexual arousal, and interference with the cross-dressing does not result in intense frustration.

Fetishism is not diagnosed when sexual arousal by nonhuman objects is limited to articles of female clothing used in cross-dressing.

Diagnostic criteria for Transvestism

- A. Recurrent and persistent cross-dressing by a heterosexual male.
- B. Use of cross-dressing for the purpose of sexual excitement, at least initially in the course of the disorder.
- C. Intense frustration when the cross-dressing is interfered with.
- D. Does not meet the criteria for Transsexualism.

302.10 Zoophilia

The essential feature is the use of animals as a repeatedly preferred or exclusive method of achieving sexual excitement. The animal may be the object of intercourse or may be trained to sexually excite the human partner by licking or rubbing. Usually the preferred animal is one with which the individual had contact during childhood, such as a household pet or farm animal. The animal is preferred no matter what other forms of sexual outlet are available.

Age at onset. No information.

Course. Initially in the course of the disorder there may also be sexual arousal by humans. As time progresses, however, the animal becomes the most powerful sexual stimulus. This usually occurs by early adulthood and the course then becomes chronic.

Differential diagnosis. Nonpathological sexual activity with animals may occur because of the unavailability of suitable human partners or as a form of sexual experimentation. In such instances the use of animals is not the consistently preferred method of achieving sexual excitement.

Diagnostic criteria for Zoophilia

The act or fantasy of engaging in sexual activity with animals is a repeatedly preferred or exclusive method of achieving sexual excitement.

302.20 Pedophilia

The essential feature is the act or fantasy of engaging in sexual activity with prepubertal children as a repeatedly preferred or exclusive method of achieving sexual excitement. The difference in age between the adult with this disorder and the prepubertal child is arbitrarily set at ten years or more. For late adolescents with the disorder, no precise age difference is specified; and clinical judgment must be used, the sexual maturity of the child as well as the age difference being taken into account.

Adults with the disorder are oriented toward children of the other sex twice as often as toward children of the same sex. The sexual behavior of these two groups is different. Heterosexually oriented males tend to prefer eight-to-ten year-old girls, the desired sexual activity usually being limited to looking or touching. Most incidents are initiated by adults who are in the intimate interpersonal environment of the child. Homosexually oriented males tend to prefer slightly older children. The percentage of couples in this group who know each other only casually is higher than in the heterosexually oriented group. Individuals with undifferentiated sexual object preference tend to prefer younger children than either of the other two groups.

Most individuals oriented homosexually have not been married, whereas most individuals oriented heterosexually either have been or are married.

Age at onset. The disorder may begin at any time in adulthood; most frequently it begins in middle age.

Course. The course is unknown, although homosexually oriented Pedophilia tends to be chronic. The severity of the condition often fluctuates with psychosocial stress. The recidivism rate for homosexually oriented Pedophilia is second only to that for Exhibitionism, and ranges from 13% to 28% of those apprehended, roughly twice that of heterosexually oriented Pedophilia.

Differential diagnosis. Isolated sexual acts with children do not warrant the diagnosis of Pedophilia. Such acts may be precipitated by marital discord, recent loss, or intense loneliness. In such instances the desire for sex with a child may be understood as a substitute for a preferred but unavailable adult. In **Mental Retardation**, **Organic Personality Syndrome**, **Alcohol Intoxication**, or **Schizophrenia** there may be a decrease in impulse control, particularly in the elderly, that in rare instances leads to isolated sexual acts with children. However, sexual activity with children is generally not the consistently preferred method for achieving sexual excitement.

In **Exhibitionism** exposure may be to a child, but the act is not a prelude to further sexual activity with the child.

Sexual Sadism may, in extremely rare instances, be associated with Pedophilia, in which case both diagnoses are warranted.

Diagnostic criteria for Pedophilia

- A. The act or fantasy of engaging in sexual activity with prepubertal chil-

dren is a repeatedly preferred or exclusive method of achieving sexual excitement.

B. If the individual is an adult, the prepubertal children are at least ten years younger than the individual. If the individual is a late adolescent, no precise age difference is required, and clinical judgment must take into account the age difference as well as the sexual maturity of the child.

302.40 Exhibitionism

The essential feature is repetitive acts of exposing the genitals to an unsuspecting stranger for the purpose of achieving sexual excitement, with no attempt at further sexual activity with the stranger. The wish to surprise or shock the observer is often consciously perceived or close to conscious awareness, but these individuals are usually not physically dangerous to the victim. Sometimes the individual masturbates while exposing himself. The condition apparently occurs only in males, and the victims are female children or adults.

Age at onset and course. The disorder may first occur at any time from preadolescence to middle age, although it rarely begins at either end of the age spectrum. The peak age at onset is the middle 20s, with a smaller peak in mid-puberty.

Few arrests are made in the older age groups, which suggests that the condition becomes less severe after age 40.

Differential diagnosis. Repeated exposure without experiencing sexual excitement from the act is engaged in by a small number of individuals. They should not receive the diagnosis of Exhibitionism since it is likely that such individuals suffer from another disorder.

When exposure occurs in **Pedophilia** it is a prelude to sexual activity with the child.

Diagnostic criteria for Exhibitionism

Repetitive acts of exposing the genitals to an unsuspecting stranger for the purpose of achieving sexual excitement, with no attempt at further sexual activity with the stranger.

302.82 Voyeurism

The essential feature is repetitive looking at unsuspecting people, usually strangers, who are either naked, in the act of disrobing, or engaging in sexual activity, as the repeatedly preferred or exclusive method of achieving sexual excitement. The act of looking ("peeping") is for the purpose of achieving sexual excitement, and no sexual activity with the person is sought. Orgasm, usually produced by masturbation, may occur during the voyeuristic activity, or later in response to the memory of what the individual has witnessed. Often these individuals enjoy thinking about the observed individuals' being helpless and feeling humiliated if

they knew they were being seen. In its severe form, peeping constitutes the exclusive form of sexual activity.

Age at onset. The first voyeuristic act is likely to occur in early adulthood.

Course. The course tends to be chronic.

Differential diagnosis. Normal sexual activity often includes sexual excitement from observing nudity, undressing, or sexual activity. However, it is not with an unsuspecting partner, and it is usually a prelude to further sexual activity. **Watching pornography**, filmed or live, causes sexual excitement. However, the people who are being observed are willingly in view, even though in fantasy the observer may *imagine* (but knows better) that the people are unsuspecting.

Diagnostic criteria for Voyeurism

- A. The individual repeatedly observes unsuspecting people who are naked, in the act of disrobing, or engaging in sexual activity and no sexual activity with the observed people is sought.
- B. The observing is the repeatedly preferred or exclusive method of achieving sexual excitement.

302.83 Sexual Masochism

The essential feature is sexual excitement produced in an individual by his or her own suffering. The diagnosis of Sexual Masochism is warranted under either of two conditions:

- (1) A preferred or exclusive mode of producing sexual excitement is to be humiliated, bound, beaten, or otherwise made to suffer.
- (2) The individual has intentionally participated in an activity in which he or she was physically harmed or his or her life was threatened in order to produce sexual excitement, which did occur. A single well-documented episode is sufficient to make the diagnosis.

Age at onset. Masochistic sexual fantasies are likely to have been present in childhood. However, the age when masochistic activities with partners first begin is variable, but is commonly by early adulthood.

Course. The disorder is usually chronic. Self-mutilation, if engaged in, is likely to be repeated. Some individuals with the disorder may for many years engage in masochistic acts without a need to increase the potential for self-harm. Others, however, either because of an increased need or a diminished capacity for restraint, increase the severity of the masochistic acts over time, or during periods of stress, which may result in death.

Differential diagnosis. Masochistic fantasies of being bound, beaten, raped,

or otherwise humiliated may facilitate sexual excitement in some individuals; without such fantasies, they find sexual arousal inadequate. The diagnosis of Sexual Masochism is made only if the individual engages in masochistic sexual acts, not merely fantasies. (If the need for masochistic fantasies is considered clinically significant, it may be diagnosed as Psychosexual Disorder Not Elsewhere Classified.) Some individuals have experimented with bondage or have occasionally experienced erotic excitement as a result of unintentionally having been humiliated, but these situations are also not sufficient for diagnosing this disorder. **Masochistic personality traits**, such as the need to be disappointed or humiliated, are distinguished from Sexual Masochism by the fact that they are not associated with sexual excitement.

Diagnostic criteria for Sexual Masochism

Either (1) or (2):

- (1) a preferred or exclusive mode of producing sexual excitement is to be humiliated, bound, beaten, or otherwise made to suffer
- (2) the individual has intentionally participated in an activity in which he or she was physically harmed or his or her life was threatened, in order to produce sexual excitement

302.84 Sexual Sadism

The essential feature is the infliction of physical or psychological suffering on another person in order to achieve sexual excitement.

The diagnosis of Sexual Sadism is warranted under any of three different conditions:

- (1) On a nonconsenting partner, the individual has repeatedly and intentionally inflicted psychological or physical suffering in order to achieve sexual excitement.
- (2) With a consenting partner a repeatedly preferred or exclusive mode of achieving sexual excitement combines humiliation with simulated or mildly injurious bodily suffering.
- (3) On a consenting partner bodily injury that is extensive, permanent, or possibly mortal is inflicted in order to achieve sexual excitement.

Age at onset. Sadistic sexual fantasies are likely to have been present in childhood. The age at onset of sadistic activities is also variable, but is commonly by early adulthood.

Course. The condition is usually chronic in its extreme form. When Sexual Sadism is practiced with nonconsenting partners, the activity is likely to be repeated until the individual is apprehended.

Some individuals with the disorder may for many years engage in sadistic acts without a need to increase the potential for inflicting serious physical damage. Others, however, either because of an increased need or a diminished capacity for restraint, increase the severity of the sadistic acts over time or during

periods of stress. When the disorder is severe, these individuals may rape, torture, or kill their victims.

Familial pattern. Although brutality commonly occurs in the families of individuals with this disorder, there is no information on whether Sexual Sadism is more common in family members.

Differential diagnosis. Rape or other sexual assault may be committed by individuals with this disorder. In such instances the suffering inflicted on the victim increases the sexual excitement of the assailant. However, it should not be assumed that all or even many rapists are motivated by Sexual Sadism. Often a rapist is not motivated by the prospect of inflicting suffering, and may even lose sexual desire as a consequence. These represent two ends of a spectrum, and for cases falling in the middle, it may be very difficult for the clinician to decide if the diagnosis of Sexual Sadism is warranted.

Diagnostic criteria for Sexual Sadism

One of the following:

- (1) on a nonconsenting partner, the individual has repeatedly intentionally inflicted psychological or physical suffering in order to produce sexual excitement
- (2) with a consenting partner, the repeatedly preferred or exclusive mode of achieving sexual excitement combines humiliation with simulated or mildly injurious bodily suffering
- (3) on a consenting partner, bodily injury that is extensive, permanent, or possibly mortal is inflicted in order to achieve sexual excitement

302.90 Atypical Paraphilia

This is a residual category for individuals with Paraphilias that cannot be classified in any of the other categories. Such conditions include: Coprophilia (feces); Frotteurism (rubbing); Klismaphilia (enema); Mysophilia (filth); Necrophilia (corpse); Telephone Scatologia (lewdness); and Urophilia (urine).

PSYCHOSEXUAL DYSFUNCTIONS

The essential feature is inhibition in the appetitive or psychophysiological changes that characterize the complete sexual response cycle. Ordinarily this diagnostic category will be applied only when the disturbance is a major part of the clinical picture, although it may not be part of the chief complaint. The diagnosis is not made if the sexual dysfunction is attributed entirely to organic factors, such as a physical disorder or a medication, or if it is due to another Axis I mental disorder.

The complete sexual response cycle can be divided into the following phases:

1. *Appetitive*. This consists of fantasies about sexual activity and a desire to have sexual activity.

2. *Excitement*. This consists of a subjective sense of sexual pleasure and accompanying physiological changes. The major change in the male consists of penile tumescence leading to erection. In addition, there is the appearance of Cowper's gland secretion. The major changes in the female consist of vasocongestion generalized in the pelvis with vaginal lubrication and swelling of the external genitalia. In addition there are the development of the orgasmic platform, which is the narrowing of the outer third of the vagina by increased pubococcygeal muscle tension and vasocongestion; vasocongestion of the labia minora; breast tumescence; and lengthening and widening of the inner two-thirds of the vagina.

3. *Orgasm*. This consists of a peaking of sexual pleasure, with release of sexual tension and rhythmic contraction of the perineal muscles and pelvic reproductive organs. In the male there is the sensation of ejaculatory inevitability, which is followed by emission of semen, caused by contractions of the prostate, seminal vesicles, and urethra. In the female there are contractions, not always subjectively experienced as such, of the wall of the outer third of the vagina. In both the male and the female there is often generalized muscular tension or contractions, such as involuntary pelvic thrusting.

4. *Resolution*. This consists of a sense of general relaxation, well-being, and muscular relaxation. During this phase men are physiologically refractory to further erection and orgasm for a period of time. In contrast, women may be able to respond to additional stimulation almost immediately.

Inhibitions in the response cycle may occur at one or more of these phases, although inhibition in the resolution phase is rarely of primary clinical significance. Whenever more than one Psychosexual Dysfunction is present, they should all be recorded, in the order of clinical significance.

The particular manifestations of each of the Psychosexual Dysfunctions are noted in the diagnostic criteria. In most instances there will be a disturbance in both the subjective sense of pleasure or desire and objective performance. More rarely there may be subjective disturbance alone, without any objective signs of dysfunction, or, conversely, inhibition in performance without any acknowledged subjective distress.

In specifying diagnostic criteria, no attempt is made to require a minimum proportion or type of sexual encounter in which the dysfunction must occur to warrant a diagnosis. This judgment has to be made by the clinician, who must take into account various factors such as frequency, chronicity, subjective distress, and effect on other areas of functioning. The phrase "recurrent and persistent" in the diagnostic criteria is a shorthand method of designating the need for such a clinical judgment.

All of the dysfunctions may be lifelong or acquired (developing after a period of normal functioning), generalized or situational (limited to certain situations or with certain partners), and total or partial (degree or frequency of dis-

turbance). Although in most instances the dysfunctions occur during sexual activity with a partner, in some cases it may be appropriate to identify dysfunctions that occur during masturbation.

Associated features. Frequently there are no other obvious signs of disturbance. This is particularly the case in Inhibited Sexual Desire, since it does not necessarily involve impairment in performance. In other cases there may be a vague sense of not living up to some ill-defined concept of normality, or there may be a variety of complaints, such as depression, anxiety, guilt, shame, frustration, and somatic symptoms. Almost invariably a fear of failure and the development of a "spectator" attitude (self-monitoring), with extreme sensitivity to the reaction of the sexual partner, are present. This may further impair performance and satisfaction and lead to secondary avoidance of sexual activity and impaired communication with the sexual partner.

Age at onset. The most common age at onset is early adult life, although Premature Ejaculation more commonly begins with the first sexual encounters. The most common age of clinical presentation is late 20s and early 30s, a few years after establishment of a sustained sexual relationship. However, the first appearance may be later in adult life, particularly with Inhibited Sexual Excitement in the male.

Course. The course is extremely variable. As previously noted, all of the dysfunctions may be lifelong or acquired (developing after a period of normal functioning). They may be limited to a single short-lived episode or a recurrent pattern of episodic dysfunction. Inhibited Sexual Desire may develop as a reaction to any of the other Psychosexual Dysfunctions.

Impairment. Psychosexual Dysfunctions, even when severe, are not associated with impairment in occupational functioning, but the relationship with a sexual partner may suffer.

Complications. The major complications consist of disrupted marital or other sexual relationships.

Predisposing factors. There appears to be a positive but slight correlation between certain personality traits and psychopathology in general and the presence of one or more Psychosexual Dysfunctions. Histrionic traits in women frequently are associated with Inhibited Sexual Excitement and Inhibited Orgasm. Compulsive traits in men frequently are associated with Inhibited Sexual Desire and Inhibited Sexual Excitement. Anxiety appears to predispose to the development of Premature Ejaculation. Any negative attitude toward sexuality, due to particular experiences, internal conflicts, or adherence to rigid subcultural values, predisposes to the Psychosexual Dysfunctions.

Prevalence. Although the exact prevalence is not known, most of these disorders are believed to be common, particularly in their milder forms.

Sex ratio. The sex ratio varies for the particular dysfunction. Inhibited Sexual Desire and Inhibited Orgasm are more common in females. Premature Ejaculation, as defined, is restricted to men. Functional Vaginismus, by definition, is restricted to women. Although Functional Dyspareunia is defined so that it can occur in males, it rarely does.

Familial pattern. No information.

Differential diagnosis. When a **physical disorder** partially accounts for the symptoms of a Psychosexual Dysfunction, provided that to some extent psychological factors are also contributing to the disturbance, both diagnoses should be given (the physical disorder is recorded on Axis III). For example, Inhibited Sexual Excitement judged to be partly secondary to diabetes can be diagnosed if it is judged that it is partly secondary to performance anxiety as well. The measurement of nocturnal penile tumescence associated with REM sleep is a useful diagnostic technique for evaluating the degree to which a physical disorder is etiologically related to the disturbance. When the disturbed sexual performance is chronic, unvarying over time, and independent of situation, this also suggests that a physical disorder may be etiologically related to the disturbance. In many instances the underlying physical disorder may not have been previously diagnosed.

If **another Axis I mental disorder**, for example, Major Depression, is the primary cause of a disturbance in sexual functioning, such as loss of sexual desire, a Psychosexual Dysfunction should not be diagnosed. However, in some instances it will not be clear whether the disturbance in sexual functioning antedates the other mental disorder (in which case it should also be diagnosed) or whether it is secondary to the other mental disorder (in which case it should not be diagnosed). Frequently a **Personality Disorder** may coexist with a Psychosexual Dysfunction and may even be conceptualized as etiologic. In such cases the Psychosexual Dysfunction should be recorded on Axis I and the Personality Disorder, on Axis II. If a **V code condition** such as Marital Problem or Other Interpersonal Problem is the primary cause of a disturbance in functioning, the Psychosexual Dysfunction should be diagnosed, and both conditions noted.

If there is **inadequate sexual stimulation**, in either focus, intensity, or duration, the diagnosis of Psychosexual Dysfunction involving excitement or orgasm is not made.

302.71 Inhibited Sexual Desire

Diagnostic criteria

A. Persistent and pervasive inhibition of sexual desire. The judgment of inhibition is made by the clinician's taking into account factors that affect sexual desire such as age, sex, health, intensity and frequency of sexual desire, and the context of the individual's life. In actual practice this diagnosis will rarely be made unless the lack of desire is a source of distress to either the individual or his or her partner. Frequently this category will be used in conjunction with one or more of the other Psychosexual Dysfunction categories.

B. The disturbance is not caused exclusively by organic factors (e.g., physical disorder or medication) and is not due to another Axis I disorder.

302.72 Inhibited Sexual Excitement

This has also been termed frigidity or impotence.

Diagnostic criteria

A. Recurrent and persistent inhibition of sexual excitement during sexual activity, manifested by:

In males, partial or complete failure to attain or maintain erection until completion of the sexual act, or

In females, partial or complete failure to attain or maintain the lubrication-swelling response of sexual excitement until completion of the sexual act.

B. A clinical judgment that the individual engages in sexual activity that is adequate in focus, intensity, and duration.

C. The disturbance is not caused exclusively by organic factors (e.g., physical disorder or medication) and is not due to another Axis I disorder.

302.73 Inhibited Female Orgasm

Diagnostic criteria

A. Recurrent and persistent inhibition of the female orgasm as manifested by a delay in or absence of orgasm following a normal sexual excitement phase during sexual activity that is judged by the clinician to be adequate in focus, intensity, and duration. The same individual may also meet the criteria for Inhibited Sexual Excitement if at other times there is a problem with the excitement phase of sexual activity. In such cases both categories of Psychosexual Dysfunction should be noted.

Some women are able to experience orgasm during noncoital clitoral stimulation, but are unable to experience it during coitus in the absence of manual clitoral stimulation. There is evidence to suggest that in some instances this represents a pathological inhibition that justifies this diagnosis whereas in other instances it represents a normal variation of the female sexual response. This difficult judgment is assisted by a thorough sexual evaluation, which may even require a trial of treatment.

B. The disturbance is not caused exclusively by organic factors (e.g., physical disorder or medication) and is not due to another Axis I disorder.

302.74 Inhibited Male Orgasm

Diagnostic criteria

A. Recurrent and persistent inhibition of the male orgasm as manifested by a delay in or absence of ejaculation following an adequate phase of sexual excitement. The same individual may also meet the criteria for Inhibited Sexual Excitement if at other times there is a problem with the excitement phase of sexual activity. In such cases both categories of Psychosexual Dysfunction should be noted.

B. The disturbance is not caused exclusively by organic factors (e.g., physical disorder or medication) and is not due to another Axis I disorder.

302.75 Premature Ejaculation

Diagnostic criteria

A. Ejaculation occurs before the individual wishes it, because of recurrent and persistent absence of reasonable voluntary control of ejaculation and orgasm during sexual activity. The judgment of "reasonable control" is made by the clinician's taking into account factors that affect duration of the excitement phase, such as age, novelty of the sexual partner, and the frequency and duration of coitus.

B. The disturbance is not due to another Axis I disorder.

302.76 Functional Dyspareunia

Diagnostic criteria

A. Coitus is associated with recurrent and persistent genital pain, in either the male or the female.

B. The disturbance is not caused exclusively by a physical disorder, and is not due to lack of lubrication, Functional Vaginismus, or another Axis I disorder.

306.51 Functional Vaginismus

Diagnostic criteria

A. There is a history of recurrent and persistent involuntary spasm of the musculature of the outer third of the vagina that interferes with coitus.

B. The disturbance is not caused exclusively by a physical disorder, and is not due to another Axis I disorder.

302.70 Atypical Psychosexual Dysfunction

This category is for Psychosexual Dysfunctions that cannot be classified as a specific Psychosexual Dysfunction. An example would be no erotic sensations or even complete anesthesia despite normal physiological components of sexual excitement and orgasm. Another example would be a female analogue of Premature Ejaculation.

OTHER PSYCHOSEXUAL DISORDERS

302.00 Ego-dystonic Homosexuality

The essential features are a desire to acquire or increase heterosexual arousal, so that heterosexual relationships can be initiated or maintained, and a sustained pattern of overt homosexual arousal that the individual explicitly states has been unwanted and a persistent source of distress.

This category is reserved for those homosexuals for whom changing sexual orientations is a persistent concern, and should be avoided in cases where the desire to change sexual orientations may be a brief, temporary manifestation of an individual's difficulty in adjusting to a new awareness of his or her homosexual impulses.

Individuals with this disorder may have either no or very weak heterosexual arousal. Typically there is a history of unsuccessful attempts at initiating or sustaining heterosexual relationships. In some cases no attempt has been made to initiate a heterosexual relationship because of the expectation of lack of sexual responsiveness. In other cases the individual has been able to have short-lived heterosexual relationships, but complains that the heterosexual impulses are too weak to sustain such relationships. When the disorder is present in an adult, usually there is a strong desire to be able to have children and family life.

Generally individuals with this disorder have had homosexual relationships, but often the physical satisfaction is accompanied by emotional upset because of strong negative feelings regarding homosexuality. In some cases the negative feelings are so strong that the homosexual arousal has been confined to fantasy.

Associated features. Loneliness is particularly common. In addition, guilt, shame, anxiety, and depression may be present.

Age at onset. The most common age at onset is during early adolescence when the individual becomes aware that he or she is homosexually aroused and has already internalized negative feelings about homosexuality.

Course. There is some evidence that in time many individuals with this disorder give up the yearning to become heterosexual and accept themselves as homosexuals. This process is apparently facilitated by the presence of a supportive homosexual subculture. It is not known how often the disorder, without treatment, is self-limited. However, there is a general consensus that spontaneous development of a satisfactory heterosexual adjustment in individuals who previously had a sustained pattern of exclusively homosexual arousal is rare.

The extent to which therapy is able to decrease homosexual arousal, increase heterosexual arousal, or help homosexuals become satisfied with their sexuality is disputed.

Impairment. There is generally no or only mild impairment in social functioning.

Complications. Dysthymic Disorder can be a complication.

Predisposing factors. Since homosexuality itself is not considered a mental disorder, the factors that predispose to homosexuality are not included in this section. The factors that predispose to Ego-dystonic Homosexuality are those negative societal attitudes toward homosexuality that have been internalized. In addition, features associated with heterosexuality, such as having children and socially sanctioned family life, may be viewed as desirable and incompatible with a homosexual arousal pattern.

Prevalence, sex ratio, and familial pattern. No information.

Differential diagnosis. Homosexuality that is **ego-syntonic** is not classified as a mental disorder. In addition, the attitude that "I guess life would be easier if I were heterosexual" does not warrant this diagnosis. This category is reserved for homosexuals for whom changing sexual orientations is a persistent concern. Similarly, distress resulting simply from a conflict between a homosexual and society should not be classified here.

Individuals with **Inhibited Sexual Desire** may sometimes attribute the lack of sexual arousal to "latent homosexuality." However, Ego-dystonic Homosexuality should be diagnosed only when homosexual arousal is overt, although it may be limited to fantasy.

Homosexuals who develop a Major Depression may then express self-hatred because of their sexual orientation. The diagnosis of Ego-dystonic Homosexuality should not be made if the ego-dystonic quality is judged to be only a transient symptom of a Depressive Disorder.

Diagnostic criteria for Ego-dystonic Homosexuality

A. The individual complains that heterosexual arousal is persistently absent or weak and significantly interferes with initiating or maintaining wanted heterosexual relationships.

B. There is a sustained pattern of homosexual arousal that the individual explicitly states has been unwanted and a persistent source of distress.

302.89 Psychosexual Disorder Not Elsewhere Classified

This is a residual category for disorders whose chief manifestations are psychological disturbances related to sexuality not covered by any of the other

specific categories in the diagnostic class of Psychosexual Disorders. In rare instances this category may be used concurrently with one of the specific diagnoses when both diagnoses are necessary to explain or describe the clinical disturbance.

Examples include the following:

(1) marked feelings of inadequacy related to self-imposed standards of masculinity or femininity, such as body habitus, size and shape of sex organs, or sexual performance;

(2) impaired pleasure during the normal physiological pelvic responses of orgasm;

(3) distress about a pattern of repeated sexual conquests with a succession of individuals who exist only as things to be used (Don Juanism and nymphomania);

(4) confusion about preferred sexual orientation.

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Factitious Disorders

"Factitious" means not real, genuine, or natural. Factitious Disorders are therefore characterized by physical or psychological symptoms that are produced by the individual and are under voluntary control. The sense of voluntary control is subjective, and can only be inferred by an outside observer.

The judgment that the behavior is under voluntary control is based, in part, on the patient's ability to simulate illness in such a way that he or she is not discovered. This involves decisions as to timing and concealment that require a degree of judgment and intellectual activity suggestive of voluntary control. However, these acts have a compulsive quality, in the sense that the individual is unable to refrain from a particular behavior, even if its dangers are known. They should therefore be considered "voluntary" in the sense that they are deliberate and purposeful, but not in the sense that the acts can be controlled. Thus, in Factitious Disorders, behavior under voluntary control is used to pursue goals that are involuntarily adopted.

The judgment that a particular behavior is under voluntary control is made by the exclusion of all other possible causes of the behavior. For example, an individual presenting with hematuria is found to have anticoagulants in his possession; he denies having taken them, but blood studies are consistent with the ingestion of the anticoagulants. A reasonable inference is that the individual may have voluntarily taken the medication. A single episode of such behavior could be accidental rather than intentional. Repeated episodes would justify an inference of voluntary production of the symptoms—a Factitious Disorder. The presence of factitious psychological or physical symptoms does not preclude the coexistence of true psychological or physical symptoms.

Factitious Disorders are distinguished from acts of malingering. In Malingering, the "patient" is also in voluntary control of the symptoms, but it is for a goal that is obviously recognizable with a knowledge of the environmental circumstances, rather than of his or her psychology. For example, a claim of physical illness in order to avoid jury duty, standing trial, or conscription into the military would be classified as Malingering. Similarly, for a patient in a mental hospital to simulate an exacerbation of his or her illness in order to avoid transfer to another, less desirable facility would be an act of malingering. In contrast, in a Factitious Disorder there is no apparent goal other than to assume the patient role. If the patient mentioned above were being transferred to an obviously more desirable facility, his or her simulated exacerbation of symptoms would be a Factitious Disorder. Whereas an act of malingering may, under certain circumstances, be considered adaptive, by definition a diagnosis of a Factitious Disorder always implies psychopathology, most often a severe personality disturbance.

In the past, some of the disorders classified here would have been subsumed within the category of Hysteria.

Factitious Disorders may present with psychological or physical symptoms. Chronic Factitious Disorder with Physical Symptoms, often referred to as Munchausen syndrome, is the best known and most frequently reported of the Factitious Disorders. The other two categories included in this section are Factitious Disorder with Psychological Symptoms and Atypical Factitious Disorder with Physical Symptoms.

300.16 Factitious Disorder with Psychological Symptoms

The essential feature is the voluntary production of severe psychological (often psychotic) symptoms, suggestive of mental disorder. The individual's goal is apparently to assume the "patient" role and is not otherwise understandable in light of the individual's environmental circumstances (as is the case in Malin-gering). This has also been referred to as Ganser syndrome, pseudopsychosis, or pseudodementia.

This disorder is often recognized by the pan-symptomatic complex of psychological symptoms that are presented and by the fact that the symptoms are worse when the individual is aware of being observed. Such an individual may claim memory loss (recent and remote), hallucinations (auditory and visual), and dissociative and conversion symptoms, along with suicidal ideation. The same individual may be extremely suggestible and admit to many additional symptoms asked about by the examiner. Conversely, the individual may be extremely negativistic and uncooperative to further questioning. The psychological symptoms presented are usually a representation of the person's concept of mental disorder and may not conform to any of the recognized diagnostic categories.

Associated features. "*Vorbeireden*," the symptom of giving approximate answers or talking past the point, may be present. This is to be considered when the person gives answers to questions involving intellectual functions (such as calculations) that consistently are near misses of the correct response. When asked to multiply eight times eight, such a person may answer "sixty-five." This phenomenon, however, is not specific for this disorder, and may be found in individuals with Schizophrenia or in persons without mental disorders who are exhausted or are being humorous.

Factitious Disorder with Psychological Symptoms is almost always superimposed on a severe Personality Disorder. The individual may secretly use substances for the purpose of producing symptoms that suggest nonorganic mental disorder. Stimulants (amphetamines, cocaine, or caffeine) may be used to produce restlessness or insomnia; hallucinogens (LSD, mescaline, THC) to induce altered levels of consciousness and perception; analgesics (heroin, morphine) to induce euphoria; hypnotics (barbiturates, alcohol), to induce lethargy. Combinations of the above substances can produce very bizarre presentations.

Age at onset, prevalence, and familial pattern. No information.

Sex ratio. The disorder is apparently more common in males.

Predisposing factors. Severe Personality Disorder is a predisposing factor.

Course. The course may be limited to one or more brief episodes, or may be chronic.

Impairment. Impairment tends to be severe.

Complications. Frequent hospitalizations are a complication.

Differential diagnosis. Differential diagnosis of this disorder from other mental disorders is extremely difficult. The clinician may notice that the total clinical picture is not characteristic of any recognized mental disorder. Psychological tests (e.g., projective tests, or the Bender-Gestalt) may be helpful when the responses elicited suggest a mixture of perceptual, cognitive, and intellectual impairment that is not characteristic of any mental disorder but suggests, rather, the individual's concept of mental disorder. There is the danger, however, that simulated bizarre responses will be taken at face value.

A true **Dementia** frequently has a demonstrable organic etiology or pathophysiological process. In "pseudo"-dementia there are often near-miss, approximate answers rather than gross inability to answer questions correctly, as is often the case in a Factitious Disorder.

In a true psychosis, such as **Brief Reactive Psychosis** or **Schizophreniform Disorder**, the individual's behavior on the ward generally will not differ markedly from his or her behavior in the clinician's office. In contrast, in a Factitious Disorder with psychotic features the individual may appear to respond to auditory hallucinations only when under the impression that he or she is being watched.

For a discussion of the differential diagnosis with **Malingering**, see p. 331.

Diagnostic criteria for Factitious Disorder with Psychological Symptoms

A. The production of psychological symptoms is apparently under the individual's voluntary control.

B. The symptoms produced are not explained by any other mental disorder (although they may be superimposed on one).

C. The individual's goal is apparently to assume the "patient" role and is not otherwise understandable in light of the individual's environmental circumstances (as is the case in Malingering).

Factitious Disorder with Physical Symptoms

The essential feature is the presentation of physical symptoms that are not real. The presentation may be total fabrication, as in complaints of acute abdominal pain in the absence of any such pain; self-inflicted, as in the production of abscesses by injection of saliva into the skin; an exaggeration or exacerbation

of a preexisting physical condition, as in the acceptance of a penicillin injection despite a known previous history of an anaphylactic reaction; or any combination or variation of the above.

301.51 Chronic Factitious Disorder with Physical Symptoms

The essential feature is the individual's plausible presentation of factitious physical symptoms to such a degree that he or she is able to obtain and sustain multiple hospitalizations. The individual's entire life may consist of either trying to get admitted into or staying in hospitals. Common clinical pictures include severe right lower quadrant pain associated with nausea and vomiting, dizziness and blacking out, massive hemoptysis, generalized rash and abscesses, fevers of undetermined origin, bleeding secondary to ingestion of anticoagulants, and "lupuslike" syndromes. All organ systems are potential targets, and the symptoms presented are limited only by the individual's medical knowledge, sophistication, and imagination. This disorder has also been called Münchausen syndrome.

These individuals usually present their history with great dramatic flair, but are extremely vague and inconsistent when questioned in more detail. There may be uncontrollable pathological lying, in a manner intriguing to the listener, about any aspect of the individual's history or symptomatology (*pseudologica fantastica*). These individuals often have extensive knowledge of medical terminology and hospital routines. Once admitted to a hospital they can create havoc on the ward by demanding attention from hospital staff and by non-compliance with hospital routines and regulations. After an extensive work-up of their initial chief complaints proves negative, they will often complain of other physical problems and produce more factitious symptoms. Complaints of pain and requests for analgesics are very common. Individuals with this disorder often eagerly undergo multiple invasive procedures and operations. While in the hospital they usually have few visitors. When confronted with evidence of their factitious symptoms they either deny the allegations or rapidly discharge themselves against medical advice. They will frequently be admitted to another hospital the same day. Their courses of hospitalizations often take them to numerous cities, states, countries, and even different continents. Eventually a point is usually reached at which the individual is "caught" producing factitious symptomatology; he or she is recognized by someone from a previous admission or another hospital, or other hospitals are contacted and confirm multiple prior hospitalizations for factitious symptomatology.

Associated features. Substance abuse, particularly of analgesics and sedatives, often medically prescribed, may be present.

Age at onset and course. Onset is usually in early adult life, often with a hospitalization for true physical illness. Rapidly thereafter, a pattern of successive hospitalizations begins and becomes a lifelong pattern.

Impairment. This disorder is extremely incapacitating. The course of chronic hospitalizations is obviously incompatible with the individual's maintaining

steady employment, maintaining family ties, and forming lasting interpersonal relationships.

Complications. These individuals frequently acquire a "gridiron abdomen" from the multiple surgical procedures they have undergone. Multiple hospitalizations frequently lead to iatrogenically-induced physical illness, such as scar tissue formation from unnecessary surgery, abscesses from numerous injections, and adverse drug reactions. Occasionally they will spend time in jail because of vagrancy, or assault in mental hospitals because of transfers from general hospitals when the factitious nature of their symptoms is discovered.

Predisposing factors. These may include true physical disorder during childhood or adolescence leading to extensive medical treatment and hospitalization; a grudge against the medical profession, sometimes due to previous medical mismanagement; employment in the medical field as a nurse, technician, or other paraprofessional; underlying dependent, exploitative, or masochistic personality traits; an important relationship with a physician in the past, e.g., a family member who was a physician, or seduction by a physician during childhood or adolescence.

Prevalence. Some believe the disorder is common but rarely recognized. Others believe that it is rare and that the few individuals with the disorder are being overreported because they appear to different physicians at different hospitals, often using different names.

Sex ratio. The disorder is apparently more common in males.

Familial pattern. No information.

Differential diagnosis. The major diagnostic consideration is obviously **true physical disorder**. A high index of suspicion for Chronic Factitious Disorder with Physical Symptoms should be aroused if any combination of the following is noted: pseudologica fantastica, with emphasis on the dramatic presentation; disruptive behavior on the ward, including noncompliance with hospital rules and regulations and arguing excessively with the nurses and physicians; extensive knowledge of medical terminology and hospital routines; continued use of analgesics for "pain"; evidence of multiple surgical interventions, e.g., a "gridiron abdomen" or burr holes in the skull; extensive history of traveling; few, if any, visitors while hospitalized; and a fluctuating clinical course with the rapid production of "complications" or new "pathology" once the initial work-up proves to be negative.

In **Somatoform Disorders** there are also physical complaints not due to true physical disorder. However, the symptom production is not under voluntary control, and admissions to hospitals are rarely as common as in Chronic Factitious Disorder with Physical Symptoms.

Individuals with **Malingering** may seek hospitalization by producing symptoms in attempts to obtain compensation, evade the police, or simply "get a

bed for the night." However, the goal is usually apparent, and they can "stop" the symptom when it is no longer useful to them.

Antisocial Personality Disorder is often incorrectly diagnosed on the basis of the pseudologica fantastica, the lack of close relations with others, and the occasionally associated drug and criminal histories. Antisocial Personality Disorder differs from this disorder by its earlier onset and its rare association with chronic hospitalization as a way of life.

Schizophrenia is often incorrectly diagnosed because of the bizarre life-style. However, the characteristic psychotic symptoms of Schizophrenia are not present.

Diagnostic criteria for Chronic Factitious Disorder with Physical Symptoms

A. Plausible presentation of physical symptoms that are apparently under the individual's voluntary control to such a degree that there are multiple hospitalizations.

B. The individual's goal is apparently to assume the "patient" role and is not otherwise understandable in light of the individual's environmental circumstances (as is the case in Malingering).

300.19 Atypical Factitious Disorder with Physical Symptoms

This is a residual category for Factitious Disorders with Physical Symptoms that do not fulfill the criteria for Chronic Factitious Disorder with Physical Symptoms.

Usually individuals with Atypical Factitious Disorder with Physical Symptoms do not require hospitalization. Examples include dermatitis artifacta (induced by excoriation or chemicals) and voluntary dislocation of the shoulder.